

HAIR LOSS

- If rash on scalp →80.
- Are hair follicle openings visible in area/s of hair loss?

Yes
Is hair loss patchy or generalised?

No

Patchy

Generalised

Scarring alopecia
likely

- Test for syphilis. If positive → 53.
- Does patient wear tightly-pulled ponytails, buns, braids or weaves, with hair loss along hairline or in area of braids/weave?

Yes

No



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Traction alopecia likely

- Explain cause.
- Advise to avoid tight or painful hairstyles.
- Reassure that hair will usually grow again once cause removed.
- If no better after 3 months, refer.

Are patches well-defined with healthy underlying scalp?

Yes

No: is patient a woman with thinning of hair over top of head?



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Alopecia areata likely

- Apply **betamethasone 0.1%** cream twice a day for 3 months.
- Check TSH. If abnormal, refer to doctor.
- Advise that hair may take up to 2 years to regrow.
- Refer if:
 - Extensive/multiple patches
 - No better with treatment
 - Recurrent

Yes

No



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Female pattern hair loss likely

- Check TSH and ferritin. If abnormal, refer to doctor.
- Check Hb: if < 12 (woman) or < 13 (man) → 27.
- Advise to use hair styles that may hide hair loss.
- Refer if:
 - Abnormal hair growth on face or body
 - Irregular periods or infertility in woman of child bearing age
 - Severe acne
 - Causing severe distress

- Refer if:
 - Syphilis negative
 - Syphilis positive and no improvement 3 months after syphilis treatment.

- Ask about recent possible causes:
 - Major illness or surgery
 - Major stress
 - Childbirth
 - Poor diet
 - Significant weight loss
- Review medication: sodium valproate, simvastatin and hormonal contraceptives can cause hair loss. Discuss with doctor.
- Test for syphilis. If positive → 53.
- Check TSH and ferritin. If abnormal, refer to doctor.
- Check Hb: if < 12 (woman) or < 13 (man) → 27.
- Reassure that hair will grow again once cause treated/resolved.
- Refer if:
 - Cause unclear
 - Woman with abnormal hair growth on face or body, irregular periods, infertility or severe acne.
 - No improvement 12 months after cause treated/resolved.

Refer.

If causing patient distress, refer for counselling.

If diagnosis uncertain, discuss/refer.